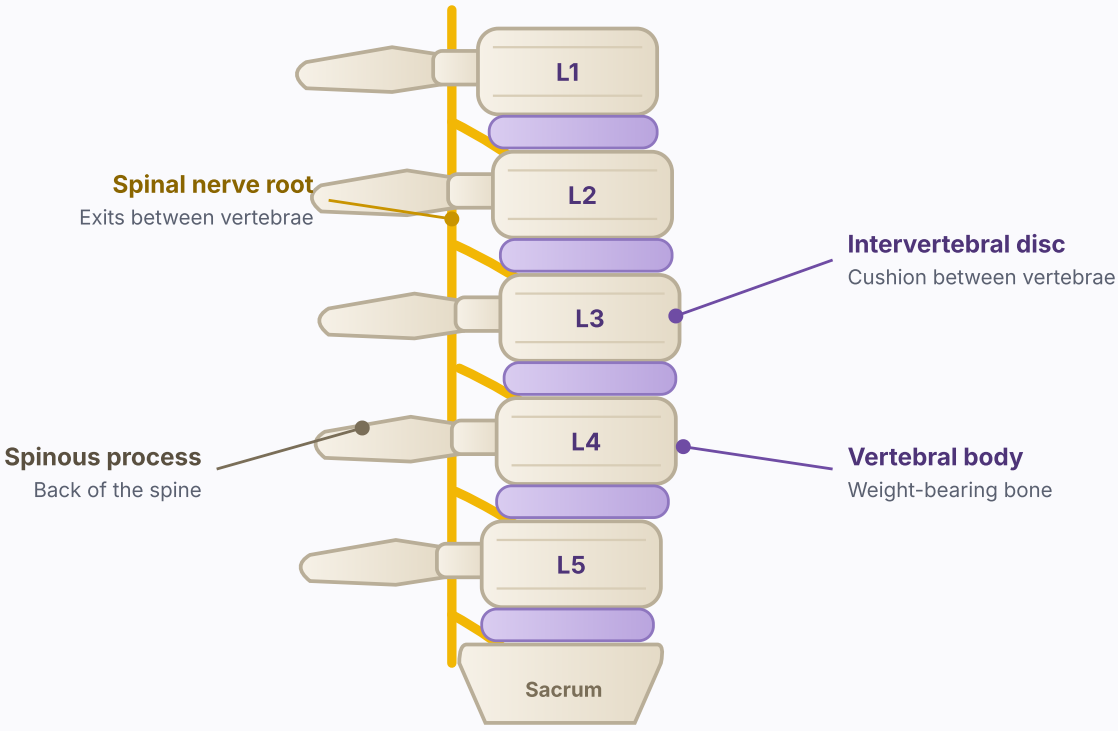
DISC CENTERS of AMERICA™

TOTAL FUZE
SPINE AND PAIN CENTER

PATIENT GUIDE · NON-SURGICAL SPINAL DECOMPRESSION

Home Care Recommendations

What to do before, during, and after your decompression program to protect your progress.



LUMBAR SPINE · SIDE VIEW (FRONT OF BODY TO THE RIGHT)

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Non-Surgical Spinal Decompression

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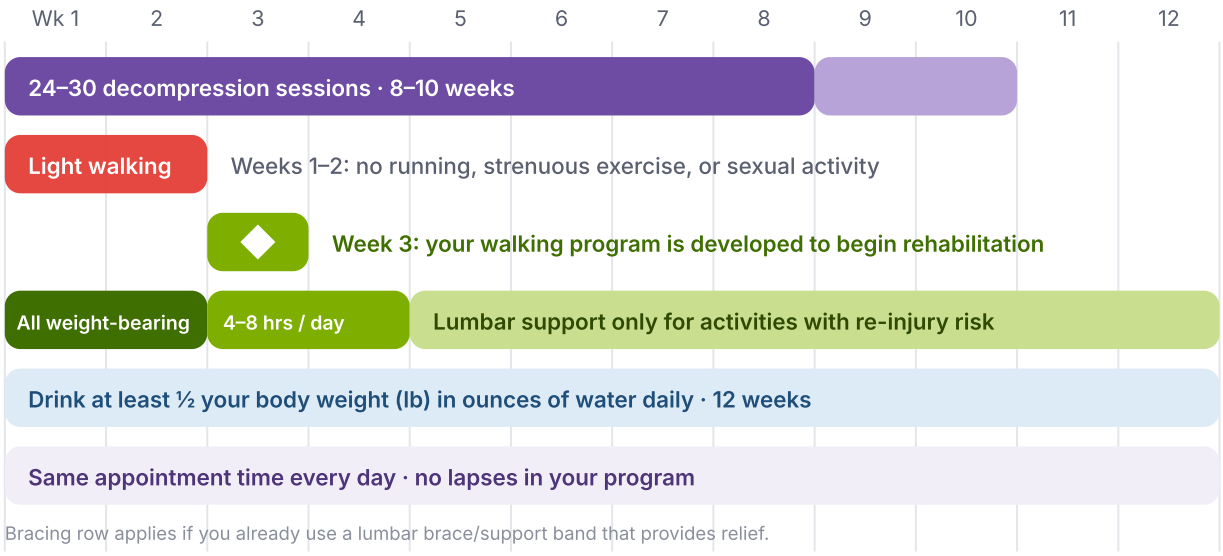
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Your Program at a Glance

Key milestones from this guide, laid out week by week. Dr. Nissa may adjust timing based on your response.



Bracing row applies if you already use a lumbar brace/support band that provides relief.

Procedures

24–30 **Non-surgical spinal decompression sessions**

8–10 **Weeks** to complete your treatment program

- **Treatment consists of 24–30 non-surgical spinal decompression sessions provided over a period of 8–10 weeks.** Our goal with this treatment is to begin the process of relieving your neck or back pain as well as restoring health to the disc-related condition that you may have.
- To consolidate healing and enhance your recovery, you will also receive **combined applications of cold therapy and electric muscle stimulation.**
- Your **rehabilitation program will begin as quickly as possible based on your pain levels.** Clinical research states that it is very difficult to achieve muscle strengthening while you are in pain; thus, the majority of your pain must be gone prior to the beginning of your rehab program.
- Depending on your response, **the order and length of the modalities and exercises will be adjusted by Dr. Nissa** to promote healing and to make your recovery experience as comfortable, satisfactory, and effective as possible.

Preparing for Treatment

The Day of Treatment

- Not missing your appointment is **CRITICAL** for your recovery. If you do ***NOT*** get treated, you will ***NOT*** get better. Consistency in your treatment program is directly related to your success. It is very important to the success of your program not to have any major lapses. If you have a business trip or long weekend scheduled, we will do our best to accommodate you; however, we do not recommend any lapses in your program.
- Arriving ***ON TIME*** for your appointment is equally ***CRITICAL***. The Decompression Table is scheduled to treat several patients per day. Each time slot is set for a particular patient and will be protected for that patient. We have reserved that time for you as well, so please arrive a few minutes early to make sure you receive your full allotted treatment.

Appointment Policies

If you **FAIL** to make your appointment, you **WILL NOT** be refunded for that missed appointment, nor will it be made up unless we have a cancellation. The Decompression Table is **BOOKED** weeks to months ahead. There are no available options for missed appointments. We acknowledge the seriousness of your debilitating condition and will guard your times, as we will for other patients.

10 min

If you are 10 minutes LATE for your appointment, your appointment is considered MISSED. Delaying your appointment affects the care and timeline for the patients who follow for the rest of that day. It would be unfair to penalize you if someone else delayed their appointment, and equally unfair to do the same to someone else should you present with that misfortune.

Same time

Your appointment time will be the same each day. Because we have a full schedule, we are unable to accommodate changing the time of your treatments. We do not enjoy presenting such a strict office policy; however, we believe you understand the necessity of this policy to ensure your success.

What to Wear for Treatment

✓ WEAR

- Jeans, chinos, khakis, or slacks
- Cotton tops such as T-shirts or sweatshirts
- Closed-toe shoes — required for treatment
- Need different clothes afterward? You are welcome to change in our restroom after treatment.

× AVOID

- **NO skirts or dresses**
- **NO shorts** of any kind · **NO** running suits
- **NO sandals**
- Slippery clothing — **NO** nylon, satin, silk, most dressy clothes
- **NO body piercings below the *NECK*** — all must be removed ***BEFORE*** you arrive

Visiting Our Spinal Decompression Suite

While we pride ourselves on being a family-oriented office, you, as well as our other patients undergoing the same treatment, are here to get better.

Adult visitors remain in the reception area during treatment

Absolutely no children in the decompression suite

All cell phones / pagers turned off in the suite

We do not like to take such a hard stance on this issue, but based on our experience, **patients who are not interrupted during their treatment are more likely to achieve better results.**

It is our goal to provide the most therapeutic atmosphere possible to enhance your healing. We encourage you to make this a period of rest and relaxation. Many of our patients enjoy listening to music, reading, or simply sleeping. Whatever you find helpful to relax, we encourage you to do so — and if there is any way we can help you achieve that relaxation, please let us know.

After Your Daily Treatment

During the initial phases of your treatment program, it is very important that you give your neck or back every opportunity to heal. Follow the recommendations below — they will help you better understand the variables that could be irritating your condition.

THE HOURS AFTER EACH DECOMPRESSION SESSION



Daily Recommendations

- ☑ **Absolutely no lifting for a minimum of 6 hours** after your decompression treatment, and avoid lifting during the program if at all possible.
- ☑ **No strenuous activity for a minimum of 6 hours** after your decompression treatment.
- ☑ **No bending over / twisting for a minimum of 3 hours** after your decompression treatment.
- ☑ **No sexual activity for the first 2 weeks** of your spinal decompression program.
- ☑ **Avoid sitting or working on the computer for extended periods** without periodic breaks to get up, stretch, and move.
- ☑ **No outside therapy or other forms of treatment related to this condition,** so that our health care team can accurately assess your progress. Please discuss with Dr. Nissa any other treatments you are currently receiving.
- ☑ **Light walking is allowed;** however, no running or strenuous exercise during the first 2 weeks of your treatment program.
- ☑ **A walking program will be developed for you in the third week** of your program to help begin the rehabilitation portion of your treatment.

Bracing Instructions

If you already have a lumbar brace / support band and it provides relief and stability, continue to use it as shown below. **Show your lumbar support to the doctors to ensure it is being used appropriately.** The lumbar support will help support your spine, limit motion in your back, and reduce pressure on the discs in your lumbar spine.

WEEKS 1–2

All weight-bearing hours

Wear it whenever you are upright and on your feet during the initial 2 weeks of the program.

WEEKS 3–4

4–8 hours per day

Especially when placing yourself in a pressurized position such as sitting, bending, lifting, etc.

WEEK 5 ON

Only when needed

Wear it only during activities that put your back at risk for re-injury.

Neck condition? If you are being treated for a neck condition, Dr. Nissa may also recommend the use of a cervical collar. If your neck feels grossly unstable or you experience intense pain with motion, ask Dr. Nissa for a recommendation.

What to Drink

While you are undergoing your Non-Surgical Spinal Decompression Treatment, we will be re-hydrating your discs. It is important that you **DRINK AT LEAST HALF your body weight in ounces of water** over the next 12 weeks (unless instructed otherwise by Dr. Nissa).

Your body weight

150 lb ÷ 2 = 

Minimum water per day

Example: a 150 lb patient drinks at least 75 oz — about 4.7 bottles of 16 oz water.

Body weight	Minimum daily water	≈ 16 oz bottles
120 lb	**60 oz**	3¾
140 lb	**70 oz**	4⅜
160 lb	**80 oz**	5
180 lb	**90 oz**	5⅝
200 lb	**100 oz**	6¼
220 lb	**110 oz**	6⅞
240 lb	**120 oz**	7½
260 lb	**130 oz**	8⅛

Bottle counts are the ounce total ÷ 16, rounded to the nearest ⅛ bottle.

MAKE IT A PLAN

- ☑ Buy a couple of cases of 16 oz bottled water so it is always on hand.
- ☑ Develop a plan to drink water consistently throughout the day.
- ☑ Reduce water intake in the later afternoon so you don't have to get out of bed 3–4 times per night.

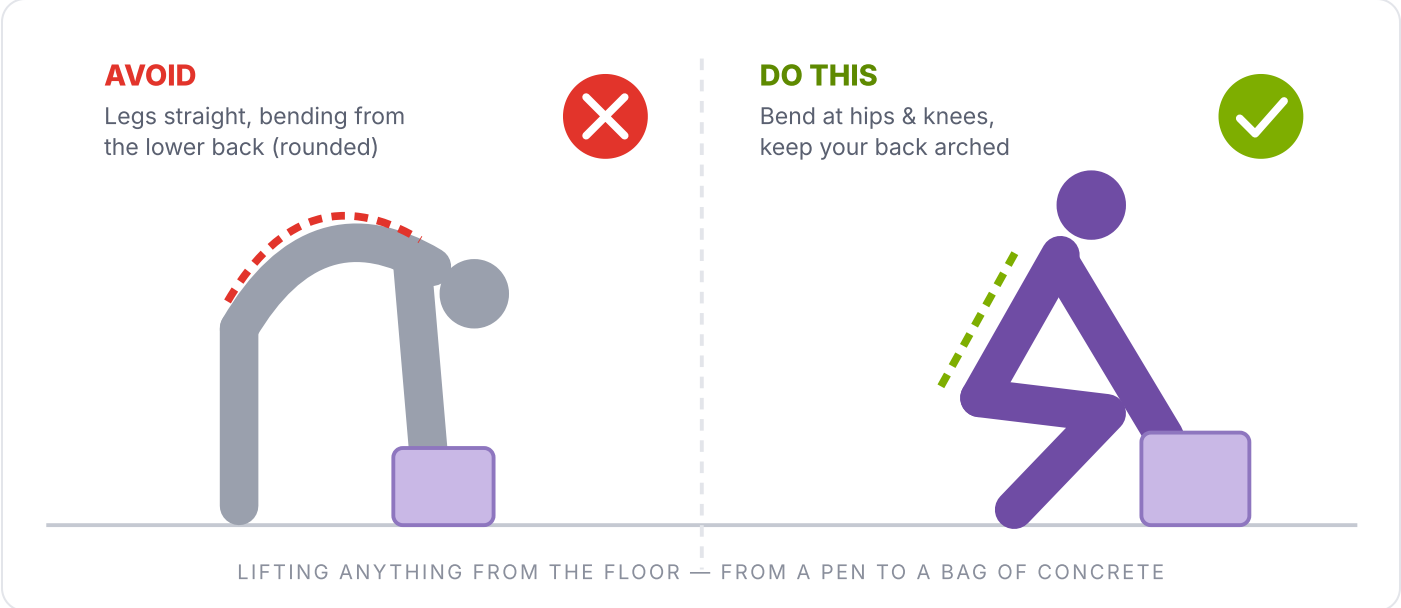
Postural Recommendations

Standing

If you have a disc-related condition, standing or being up on your feet is usually the best place to be besides lying down. When you are standing, you are most likely in a very neutral position, which places the least amount of stress on the disc-related problem.

While standing, avoid bending over any countertop, such as when you are cooking, putting on make-up in the morning, or shaving. **Try to bend at your knees and hips, not your lower back.** If you have been in pain, you know these positions are difficult, so try to avoid them or at least use the correct technique.

Also avoid any type of lifting or bending from the ground. This might be as simple as putting on your socks: instead of reaching down to put them on, **bring your foot up to you while you are sitting** to avoid the stress of bending on your lower back. If picking up anything off the floor — from a pen to a bag of concrete — **NEVER bend from your back; instead, bend from your hips and knees and always keep your back arched.**

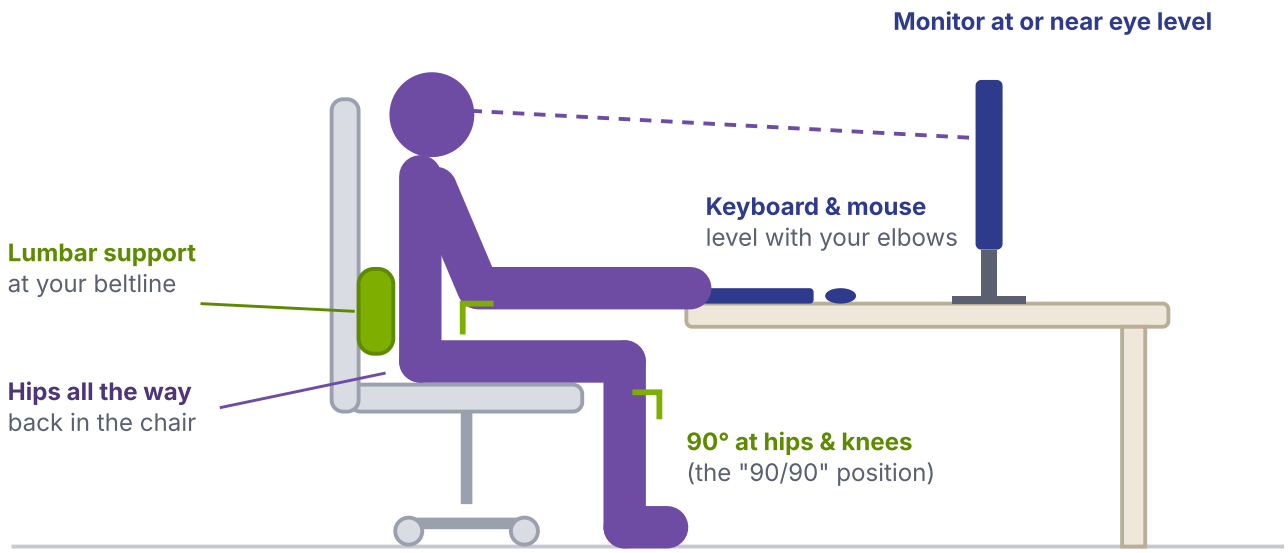


LIFTING ANYTHING FROM THE FLOOR — FROM A PEN TO A BAG OF CONCRETE

Sitting

Sitting is by far the worst position for anyone with a bulging, protruding, or herniated disc. When you sit, most people are in a slouched posture that allows their back to flatten and actually bend forward. This causes enormous pressure on the discs and further irritates the very delicate nerves.

You avoid this problem by using a **lumbar support** whenever you sit — in your car seat, your office chair, or on the couch while watching a movie. If you sit with your back flattened or slouched, you are sure to be in pain when you get up. A good lumbar support places your hips and knees in a **90/90 position** — a 90° angle at your hips and your knees — which ensures you are sitting upright.



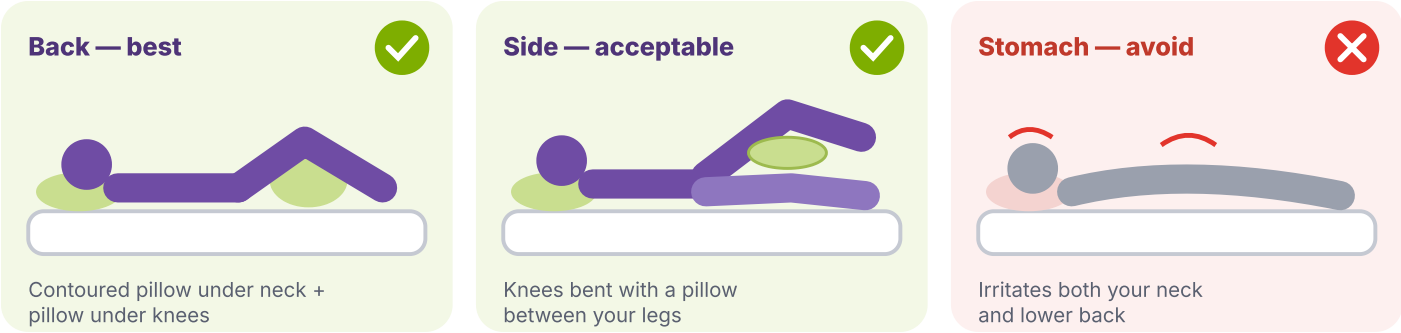
Side view. Slide your knees under the keyboard surface so you don't lean forward to reach the mouse and keyboard.

HOW TO SIT

1. Place your hips as far back in the chair as possible.
2. Place the lumbar support at your beltline so you feel it pushing into your lower back. This naturally draws your head and shoulders back into a neutral position.
3. Place your monitor at or near eye level, with your keyboard on a surface you can slide under.
4. Keep the mouse and keyboard at the same level as your elbows so you don't lean forward to use them.

Sleeping

There are only four positions to sleep in: back, side, stomach, or a hybrid of all four. **Sleeping on your back is usually the most comfortable and least stressful position** because it places your back in the most neutral position, which puts the least pressure on the discs in the lower back. A proper mattress and pillow are always recommended to provide the utmost support for both the neck and the lower back.

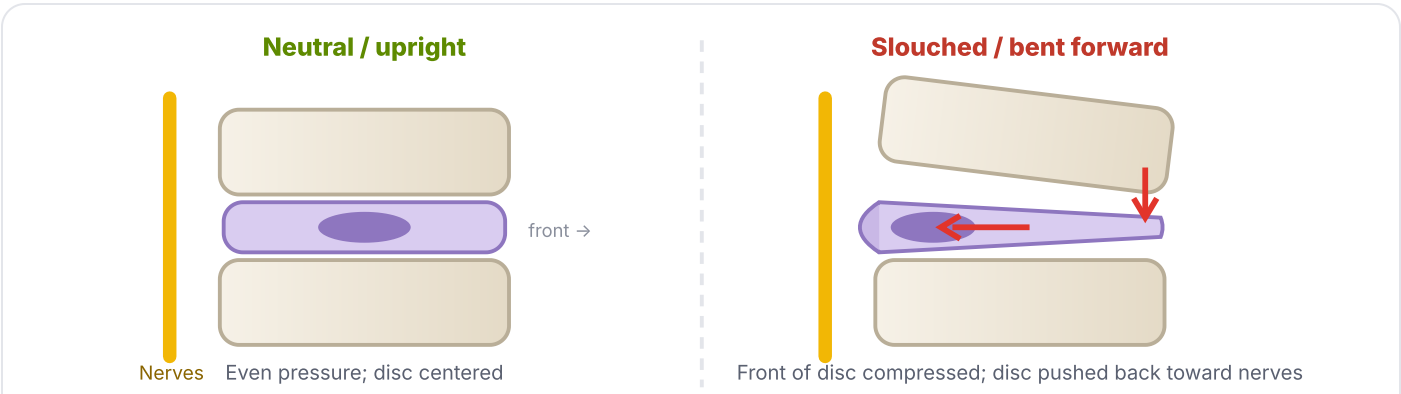


It is very important to have **one contoured, supportive pillow under your neck and a second support or pillow under your knees** to take pressure off your lower back. Sleeping on your side with a pillow between your legs is also acceptable. **Sleeping on your stomach will irritate both your neck and lower back and should be avoided.**

Special Reminder

It is very important that every person with a neck- or back-related condition understands the anatomy of their problem, how that relates to how they feel, and how they eventually heal. As you have read, it is very important to follow the strict protocols put together for your program. These protocols have taken over a decade to develop and are proven methods for getting you back on track.

When it comes to getting better, understand first that **disc-related injuries are extremely susceptible to poor positions.** Why? When you are in the wrong position, such as sitting slouched, you place pressure on the front of the discs in both the neck and the back, which causes the disc to move backwards toward the sensitive nerves and spinal cord.



Simplified side view of one spinal segment (two vertebrae and the disc between them). Back of the body on the left.

It is imperative that you avoid these positions at all costs, so that throughout your treatment program we are not in a tug-of-war with your body.